

Supplemental information

**Circulating T cell profiles associate
with enterotype signatures underlying
hematological malignancy relapses**

Nicolas Vallet, Maud Salmona, Jeanne Malet-Villemagne, Maxime Bredel, Louise Bondeelle, Simon Tournier, Séverine Mercier-Delarue, Stéphane Cassonnet, Brian Ingram, Régis Peffault de Latour, Anne Bergeron, Gérard Socié, Jérôme Le Goff, Patricia Lepage, and David Michonneau

Supplemental information

Circulating T cell profiles associate with enterotype signatures underlying hematological malignancy relapses

Nicolas Vallet, Maud Salmona, Jeanne Malet-Villemagne, Maxime Bredel, Louise Bondeelle, Simon Tournier, Séverine Mercier-Delarue, Stéphane Cassonnet, Brian Ingram, Régis Peffault de Latour, Anne Bergeron, Gérard Socié, Jérôme Le Goff, Patricia Lepage, David Michonneau

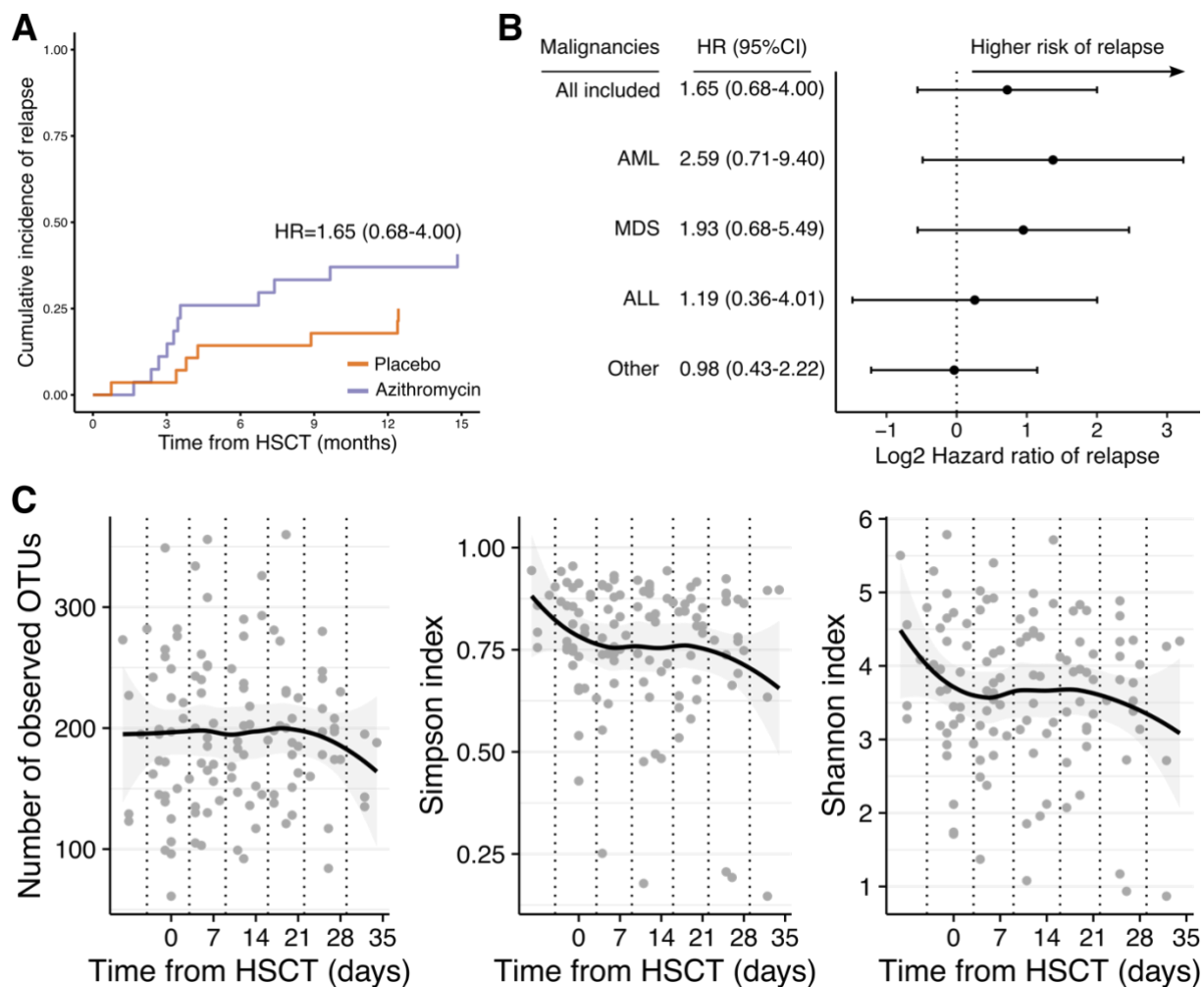


Figure S1. Relapse incidence and alpha-diversity indexes, related to Figure 1 and Figure 2. A.) Cumulative incidence of relapse with death defined as a competing risk computed with all included patients. B.) Subgroup analyses of relapse according to hematological malignancy. Using Fine and Gray model, death was defined as a competing risk. C.) Alpha-diversity indexes according to time from allogeneic hematopoietic stem cell transplantation (HSCT). Dot plots and regression line with 95% confidence interval are shown. OTU: operational taxonomic unit.

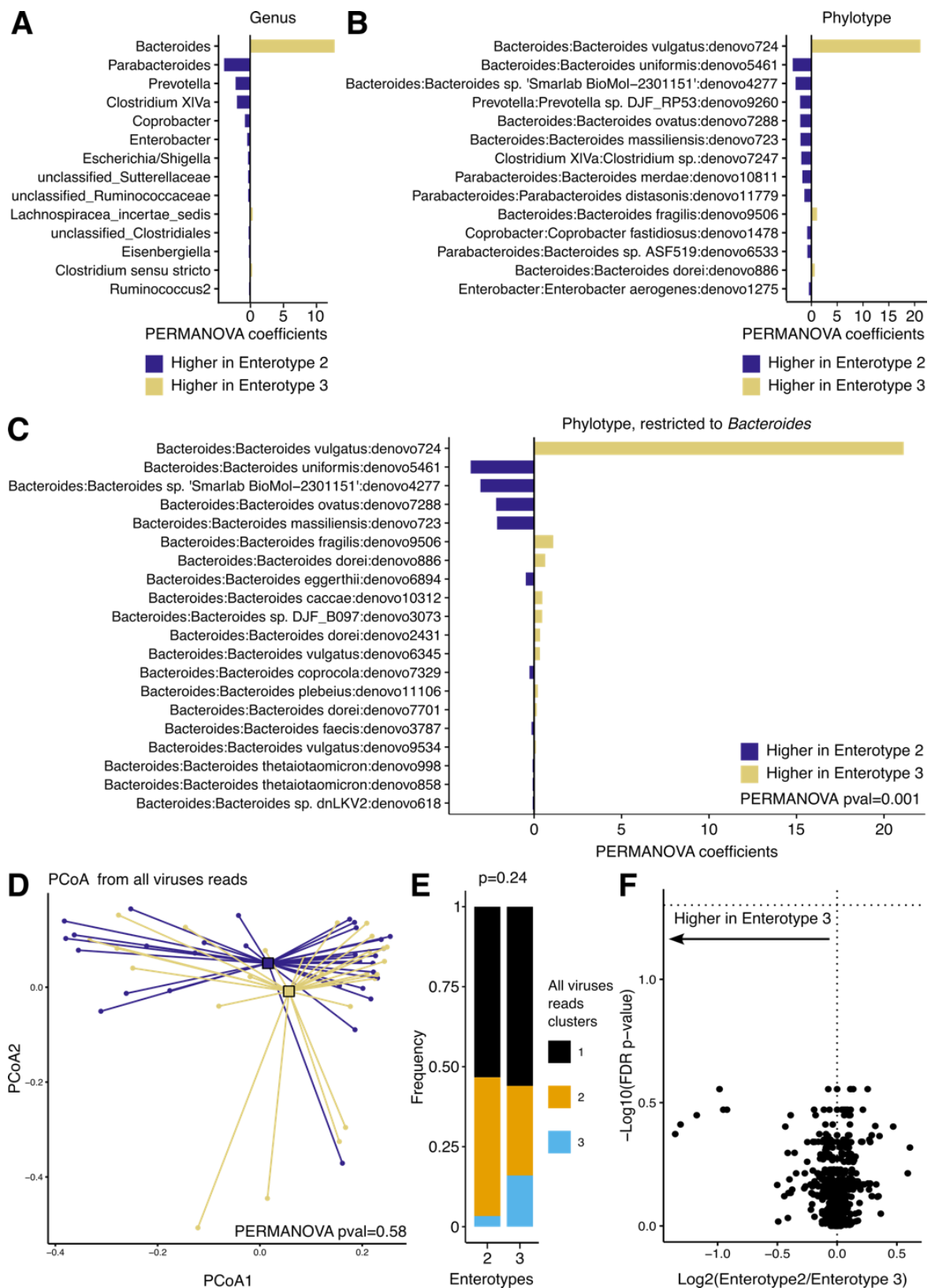


Figure S2. Comparison of the two Bacteroides enriched enterotypes (2 and 3) gut viruses and metabolites profiles, related to Figure 2. A.) Permutational multivariate analysis of variance (PERMANOVA) coefficients when comparing genus abundances from enterotypes 2 and 3. B.) PERMANOVA coefficients when comparing phylotypes abundances from enterotypes 2 and 3. C.) PERMANOVA coefficients when comparing Bacteroides phylotypes from enterotypes 2 and 3. D.) Principal co-ordinate analysis (PCoA) with UniFrac distance matrix from all viruses. Samples are colored according to their respective bacterial enterotypes 2 or enterotype 3. PERMANOVA analysis did not revealed significant differences. E.) Bar plot showing frequencies of viral enterotypes according to bacterial enterotypes 2 and enterotype 3. Fisher exact test was performed to compute P-values. F.) Volcano plot showing differences in metabolites levels between enterotype 2 and enterotype 3. Non-parametric Wilcoxon test was performed and adjusted for false discovery rate (Benjamini Hochberg).

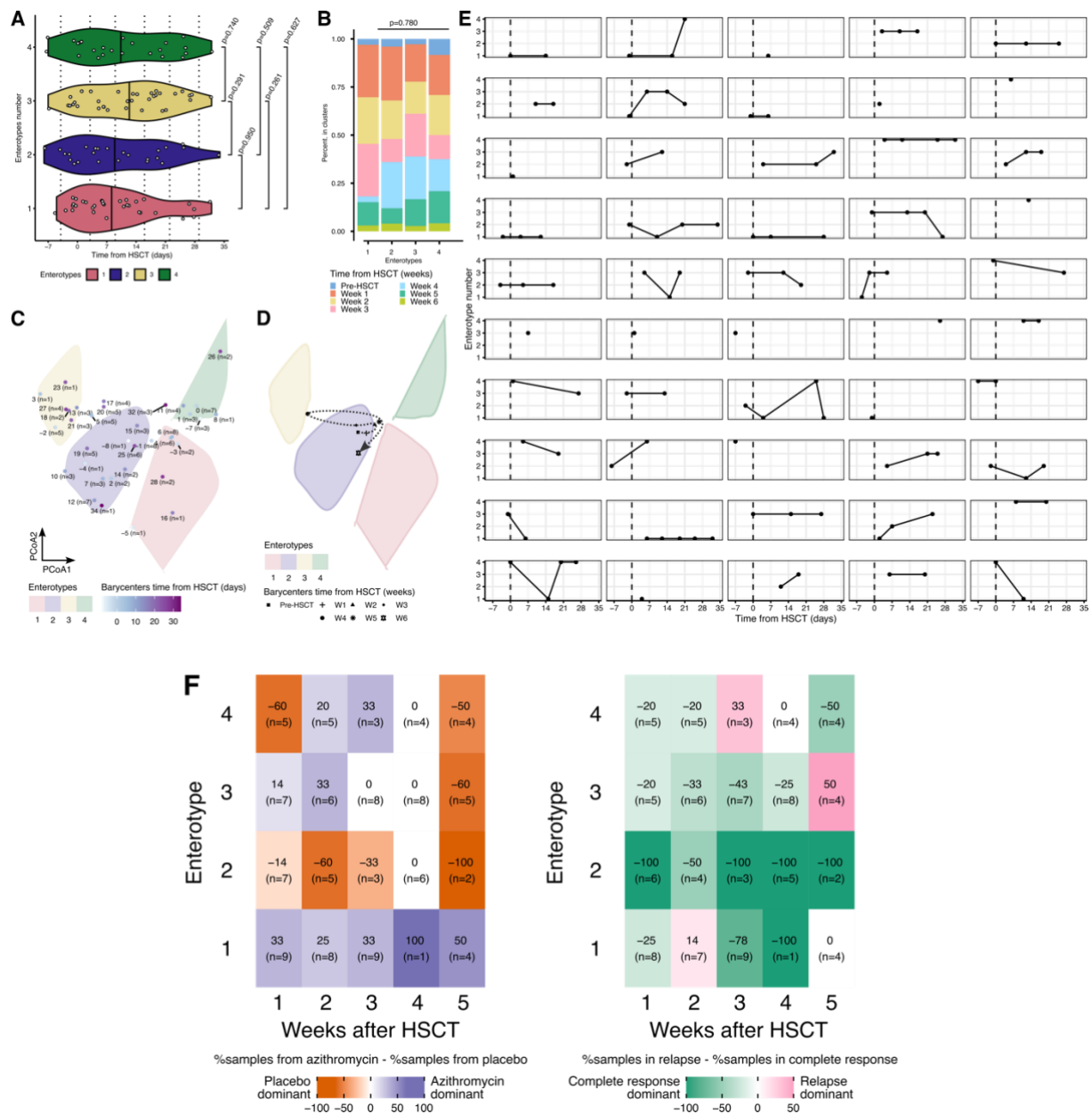


Figure S3. Enterotype according to time from allogeneic hematopoietic stem cell transplantation, related to Figure 3. A.) Comparison of time from allogeneic hematopoietic stem cell transplantation (allo-HSCT) between enterotypes, with time as a continuous variable. Dotted lines show weeks defined in Figure 1. P-value was computed with bilateral Wilcoxon test. B.) Comparison of time from allo-HSCT between enterotypes with time as a categorical variable. Weeks are defined according to sample distribution shown in Figure 1. P-value was computed with Fisher's exact test. C.) Barycenters of time from allo-HSCT and enterotypes clusters areas. Dots represents the barycenter of a day from allo-HSCT. Numbers describe the days from HSCT with the corresponding number of samples. D.) Barycenter and trajectory of samples from weeks after a HSCT as defined in Figure 1. E.) Patients individual enterotype trajectories according to time from transplantation. Each plot shows one patient sample according to time from allogeneic hematopoietic stem cell transplantation (HSCT). Vertical dashed line represents day of graft infusion. F.) Heatmaps depicting frequencies of samples in group of randomizations / complete response or relapse at 12 months according to enterotypes across time. Weeks are defined according to sample distribution shown in Figure 1.

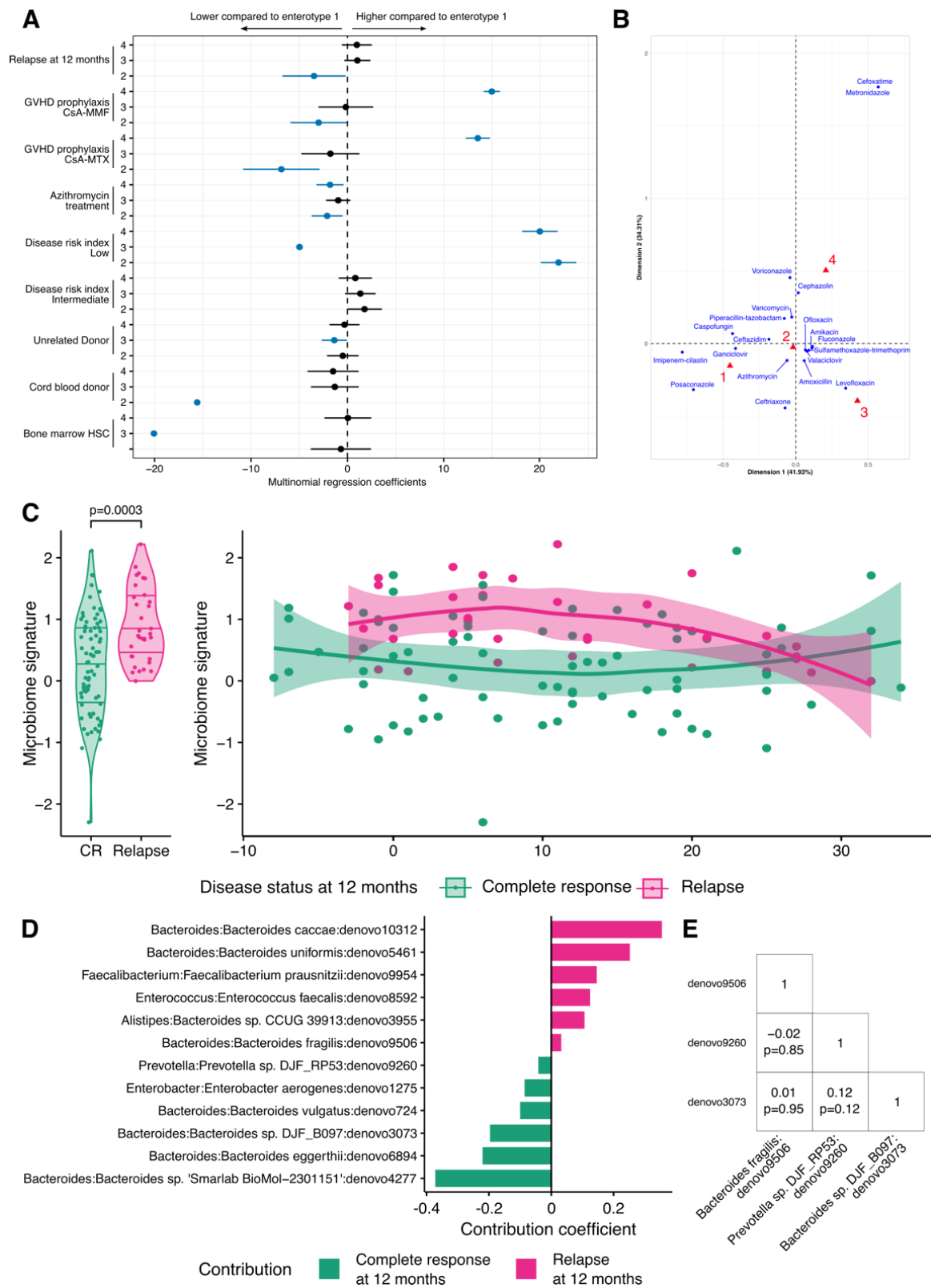


Figure S4. Characteristics of gut microbiota associated with relapse, related to Figure 3. A.) Multivariate multinomial regression models evaluating enterotypes and clinical variable association. Multivariate model includes variables associated with enterotypes (Table 1). Blue dots and line show statistically significant co-efficients. Coefficients are relative to enterotype 1 and unmentioned group in each variable. GVHD: graft-versus-host disease, CsA: cyclosporine A, MMF: mycophenolate mofetil, MTX: methotrexate, HSC: hematopoietic stem cell. B.) Correspondence analysis map of antibiotics used in the four enterotypes. Blue dots show barycenter of antibiotics used and red triangles the barycenter of enterotypes. Each barycenter represents the co-ordinates of mean distribution of the used antibiotic among the enterotypes. C.) Dynamic microbial signature associated with relapse at 12 months. Violin plot and regression curve with 95%CI showing microbial signature in samples associated with relapse and complete responses. D.) Bar plot describing taxa contribution to microbial signature. E.) Correlation analysis between phylotypes associated with relapse. Spearman co-efficients and p-value adjusted with Benjamini-Hochberg method are shown.

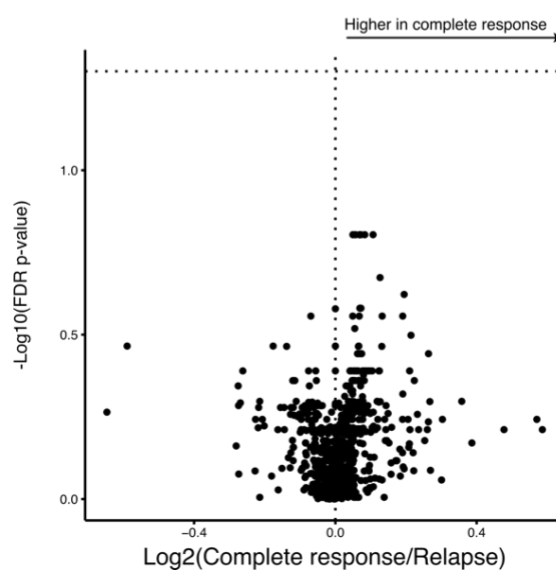
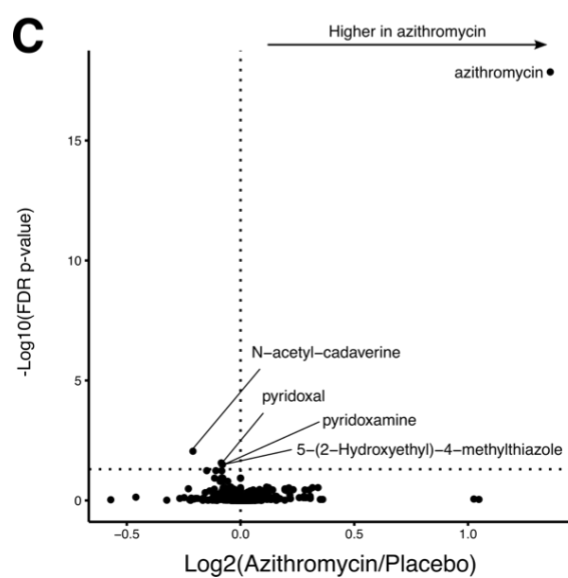
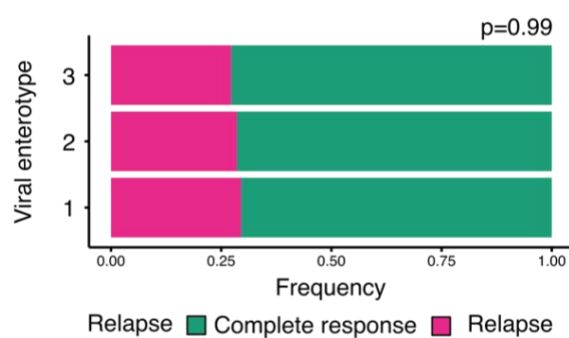
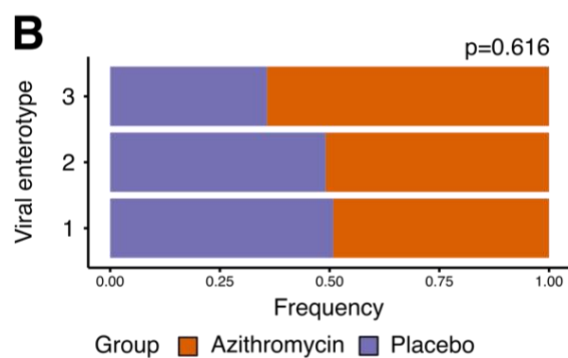
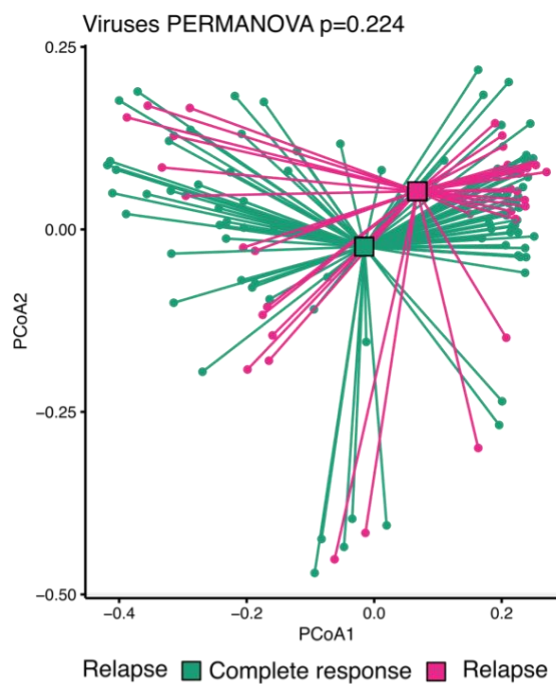
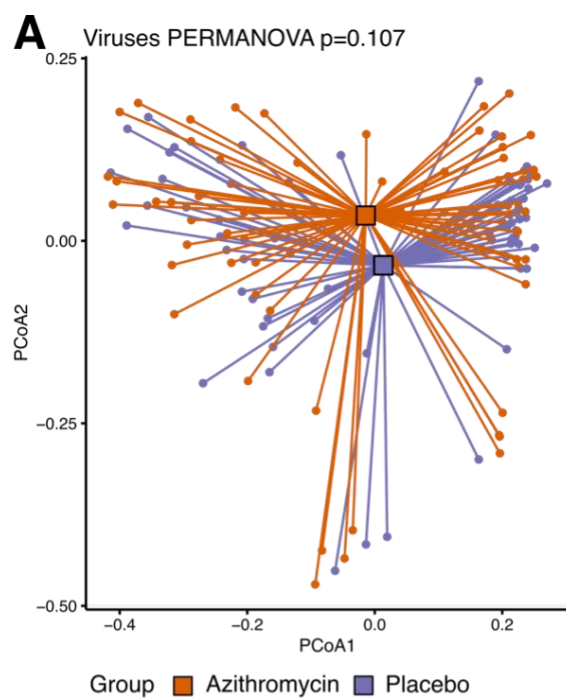
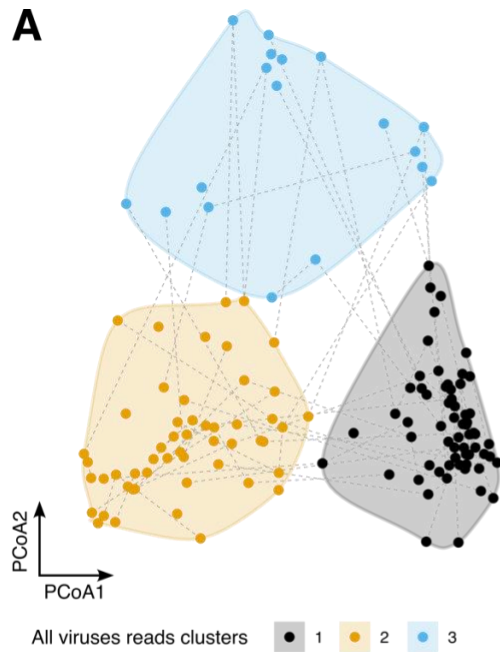
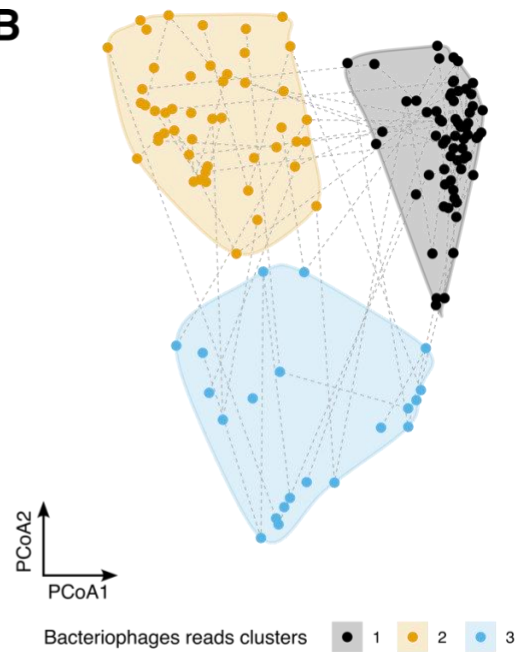


Figure S5. Comparison of gut viruses' composition and gut metabolites levels between patients in azithromycin and placebo groups and between patients in relapse or complete responses, related to Figure 3. A.) Principal co-ordinate analysis (PCoA) with UniFrac distance matrix from all viruses. Samples are colored according to azithromycin or placebo groups or according to relapse or complete responses statuses. Permutational multivariate analysis of variance (PERMANOVA) analyses did not revealed significant differences. B.) Bar plots describing frequencies of viral enterotypes according to azithromycin or placebo groups or according to relapse or complete responses statuses. Fisher exact test was performed to compute P-values. C.) Volcano plot showing differences in metabolites levels between azithromycin and placebo groups and between relapse and complete responses statuses. Non-parametric Wilcoxon test was performed and adjusted for false discovery rate (Benjamini Hochberg).

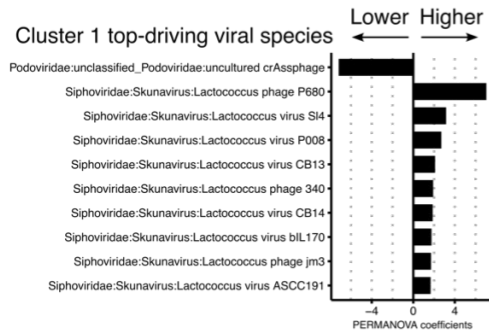
A



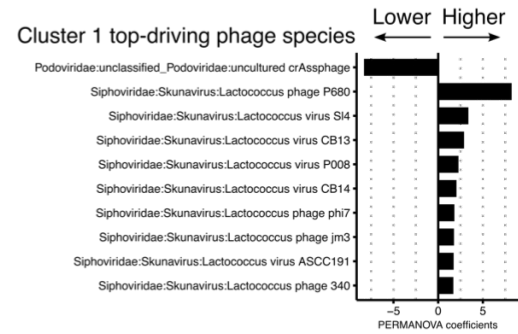
B



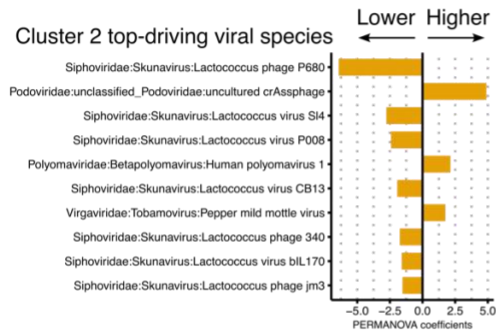
Cluster 1 top-driving viral species



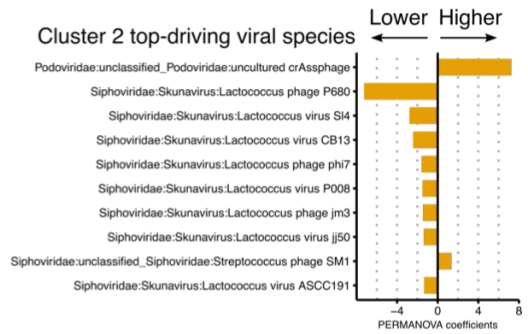
Cluster 1 top-driving phage species



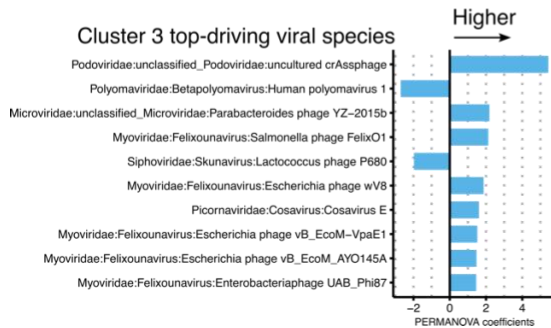
Cluster 2 top-driving viral species



Cluster 2 top-driving viral species



Cluster 3 top-driving viral species



Cluster 3 top-driving viral species

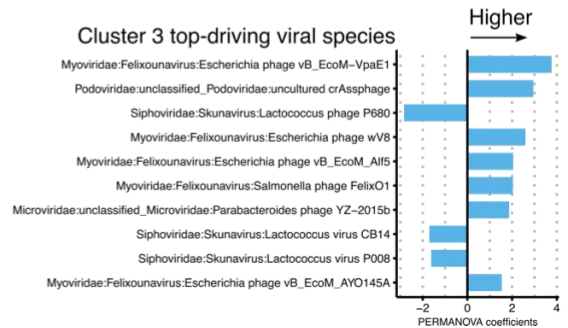


Figure S6. Principal co-ordinate analysis of viral reads and viral clusters composition, related to Figure 4. A.) Principal co-ordinate analysis (PCoA) with UniFrac distance matrix from all viruses reads. B.) Principal co-ordinate analysis with UniFrac distance matrix from bacteriophages reads only. Samples were clustered with hierarchical K-means. Driving species were computed with permutational multivariate analysis of variance (PERMANOVA).

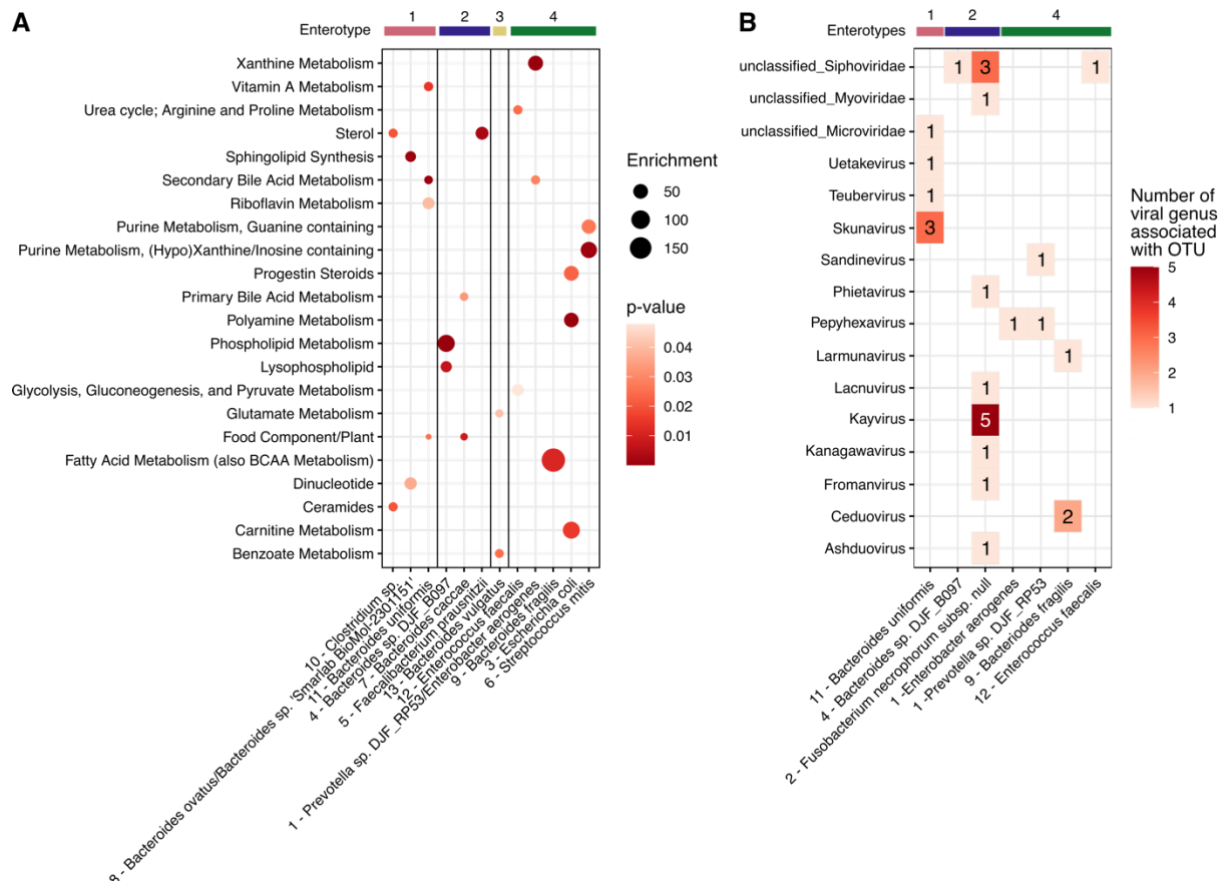


Figure S7. Metabolic pathways and viral genera are specifically associated with bacteria taxa, related to Figure 5. A.) Enriched pathways of metabolites correlated with network modules. Dot plot depicting statistically significant enriched pathways in network modules. Module numbers are associated with phylotypes species name. Phylotypes are grouped according to the enterotype they are driving (Figure S8) Enrichment was computed with over-representation method and P-value with hypergeometric test. B.) Viruses' genus associated with enterotypes top driving phylotypes. Tile plot showing number of species associated with phylotypes (OTUs) and which belongs to the same genus. Modules number are associated with phylotypes species name. Phylotypes are grouped according to the enterotype they are driving (Figure S8).

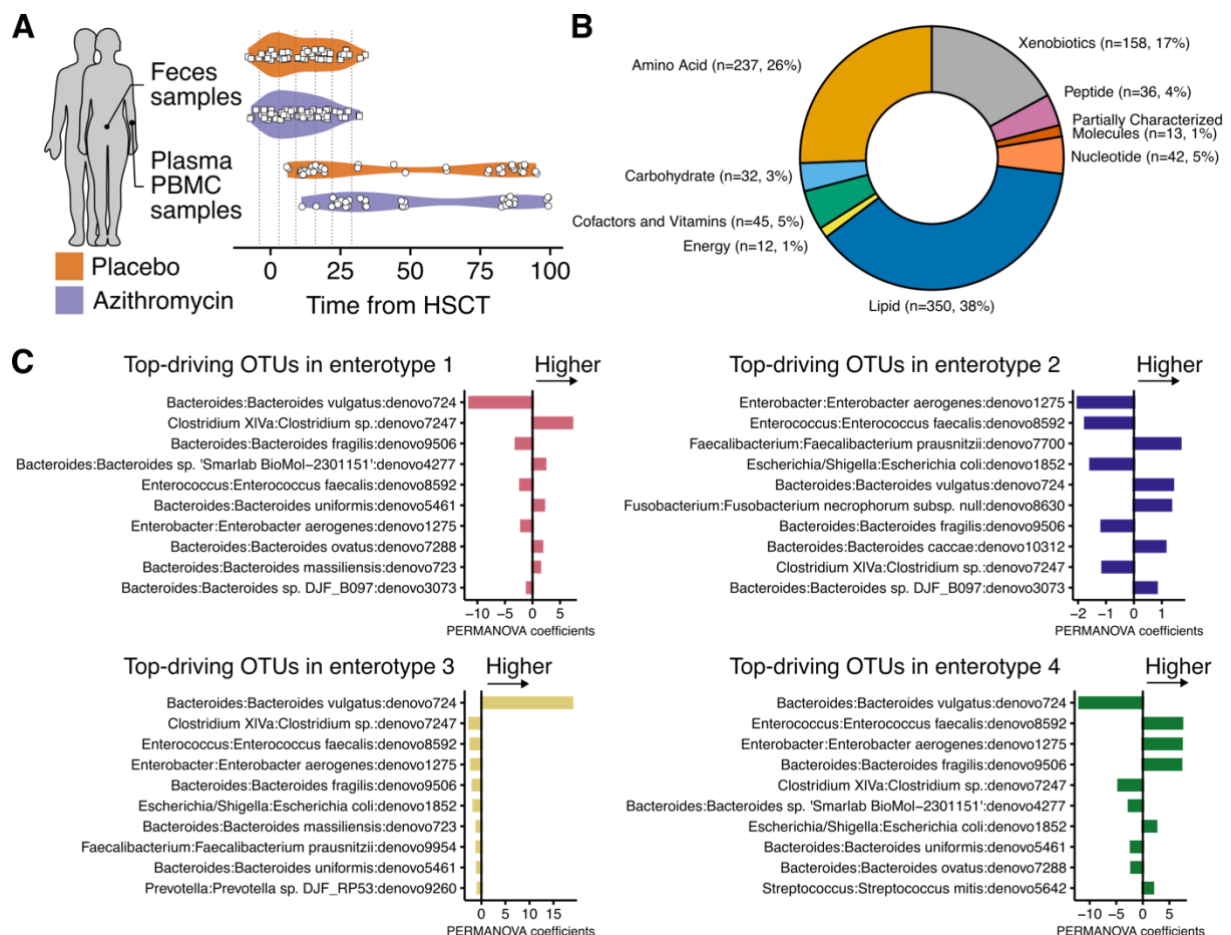


Figure S8. Supplemental material data, related to STAR Methods. A.) Violin plots depicting time from allogeneic hematopoietic stem cell transplantation (HSCT) to feces and blood samples collection. B.) Metabolites analyzed according to the corresponding superpathways. Numbers surrounding the donut plots represent the number of metabolites studied within each superpathway. C.) Top enterotypes-driving taxa. Bar plots exhibiting top 10 absolute permutational multivariate analysis of variance (PERMANOVA) coefficient of phylotypes (OTU) in each enterotype.

Table S1. Characteristics of patients included in the present study, related to Figure 1

	All patients n= 55 (148 samples)	Placebo 28 (75 samples)	Azithromycin 27 (73 samples)	p-value
Gender, n Female (%)	19 (35)	11 (39)	8 (30)	0.50
Age (years)	55 (40-62)	53 (40-60)	56 (41-64)	0.40
Diagnosis, n (%)				
Acute myeloid leukemia	11 (20)	3 (11)	8 (30)	0.08
Acute lymphoid leukemia	9 (16)	6 (21)	3 (11)	
Myelodysplastic neoplasm	13 (24)	8 (29)	5 (19)	
Non-Hodgkin lymphoma	9 (16)	7 (25)	2 (7.4)	
Hodgkin lymphoma	1 (1.8)	0 (0)	1 (3.7)	
Chronic lymphoid leukemia	3 (5.5)	0 (0)	3 (11)	
Chronic myeloid leukemia	1 (1.8)	1 (3.6)	0 (0)	
Other malignancies	8 (15)	3 (11)	5 (19)	
Disease risk index, (%)				
Low	2 (3.6)	0 (0)	2 (7.4)	0.50
Intermediate	22 (40)	12 (43)	10 (37)	
High	31 (56)	16 (57)	15 (56)	
Disease status at transplant, (%)				
CR1	10 (18)	5 (18)	5 (19)	0.99
>CR1	19 (35)	10 (36)	9 (33)	
Other	26 (47)	13 (46)	13 (48)	
Donor type, n (%)				
Related	21 (38)	12 (43)	9 (33)	0.50
Unrelated	34 (62)	16 (57)	18 (67)	
CMV serology (Donor/Recipient), n (%)				
-/-	20 (36)	11 (39)	9 (33)	0.80
-/+	12 (22)	7 (25)	5 (19)	
+/-	8 (15)	3 (11)	5 (19)	
+/+	15 (27)	7 (25)	8 (30)	
Source of stem cells, n (%)				
Peripheral blood	51 (93)	25 (89)	26 (96)	0.70
Bone marrow	2 (3.6)	2 (7.1)	0 (0)	
Cord blood	2 (3.6)	1 (3.6)	1 (3.7)	
Conditioning regimen, n (%)				
Myeloablative	16 (29)	8 (29)	8 (30)	0.99
Non-myeloablative	39 (71)	20 (71)	19 (70)	
Graft-vs-host disease prophylaxis, n (%)				
Ciclosporin-methotrexate	15 (27)	7 (25)	8 (30)	0.99
Ciclosporin-mycophenolate mofetil	38 (69)	20 (71)	18 (67)	
Post-transplant cyclophosphamide	0 (0)	0 (0)	0 (0)	
Other	2 (3.6)	1 (3.6)	1 (3.7)	
Acute GVHD, n (%)				
aGVHD	35 (64)	16 (57)	19 (70)	0.30
Death before aGVHD	11 (20)	5 (18)	6 (22)	
No event at last follow up	9 (16)	7 (25)	2 (7.4)	
Chronic GVHD, n (%)				
Mild	16 (29)	6 (21)	10 (37)	0.2
Moderate	11 (20)	7 (25)	4 (15)	
Severe	2 (3.6)	0 (0)	2 (7.4)	
No event at last follow up	26 (47)	15 (53)	11 (41)	
Bronchiolitis obliterans syndrome, n (%)				
Yes	1 (1.8)	0 (0)	1 (3.7)	NA
Relapse at 12 months, n (%)				
Relapse	15 (27)	5 (22)	10 (38)	0.20
Death before 12 months	6	5	1	
No event at 12 months	34 (69)	18 (78)	16 (62)	
Nutrition, n (%)				
Enteral	8 (15)	4 (15)	4 (15)	0.40
Oral	32 (60)	14 (52)	18 (69)	
Intravenous	13 (25)	9 (33)	4 (15)	
Unknown	2	1	1	
Antibiotics, (%)				
Amoxicilin	53 (96)	28 (100)	25 (93)	0.20
Ofloxacin	53 (96)	28 (100)	25 (93)	0.20
Piperacilline-Tazobactam	39 (71)	20 (71)	19 (70)	0.99
Ceftazidim	16 (29)	9 (32)	7 (26)	0.60
Vancomycin	24 (44)	14 (50)	10 (37)	0.30
Imipenem Cilastin	9 (16)	4 (14)	5 (19)	0.70
Amikacin	48 (87)	24 (86)	24 (89)	0.99
Ceftriaxone	1 (1.8)	0 (0)	1 (3.7)	0.50
Cephazoline	11 (20)	8 (29)	3 (11)	0.11
Sulfamethoxazole Trimethoprim	55 (100)	28 (100)	27 (100)	-
Levofloxacin	1 (1.8)	1 (3.6)	0 (0)	0.99
Metronidazole	1 (1.8)	1 (3.6)	0 (0)	0.99
Cefotaxime	1 (1.8)	1 (3.6)	0 (0)	0.99
Ganciclovir	4 (7.3)	3 (11)	1 (3.7)	0.60
Valaciclovir	54 (98)	28 (100)	26 (96)	0.50
Fluconazole	13 (24)	6 (21)	7 (26)	0.70
Caspofungine	10 (18)	7 (25)	3 (11)	0.30
Micafungine	1 (1.8)	1 (3.6)	0 (0)	0.99
Posaconazol	4 (7.3)	3 (11)	1 (3.7)	0.60
Voriconazole	9 (16)	8 (29)	1 (3.7)	0.025

Continuous variables are described with median and interquartile range and compared with bilateral Wilcoxon test. Fisher's test was used to compare frequencies. GVHD: graft-versus-host disease.

Table S2: Type of nutrition support and antibiotic used during the procedure, related to Figure 1

n=	All patients	Placebo	Azithromycin	p-value
	55 (148 samples)	28 (75 samples)	27 (73 samples)	
Nutrition, n (%)				
Enteral	8 (15)	4 (15)	4 (15)	0.40
Oral	32 (60)	14 (52)	18 (69)	
Intraveinuous	13 (25)	9 (33)	4 (15)	
Unknown	2	1	1	
Antibiotics, (%)				
Beta-lactamin				
Amoxicilin	53 (96)	28 (100)	25 (93)	0.20
Piperacilline-Tazobactam	39 (71)	20 (71)	19 (70)	0.99
Glycopeptide				
Vancomycin	24 (44)	14 (50)	10 (37)	0.30
Cephalosporin				
Ceftriaxone	1 (1.8)	0 (0)	1 (3.7)	0.50
Cephazoline	11 (20)	8 (29)	3 (11)	0.11
Ceftazidim	16 (29)	9 (32)	7 (26)	0.60
Cefotaxime	1 (1.8)	1 (3.6)	0 (0)	0.99
Quinolone				
Ofloxacin	53 (96)	28 (100)	25 (93)	0.20
Levofloxacin	1 (1.8)	1 (3.6)	0 (0)	0.99
Carbapenem				
Imipenem Cilastin	9 (16)	4 (14)	5 (19)	0.70
Sulfonamides				
Sulfamethoxazole Trimethoprim	55 (100)	28 (100)	27 (100)	NA
Nitroimidazole				
Metronidazole	1 (1.8)	1 (3.6)	0 (0)	0.99
Aminoglycoside				
Amikacin	48 (87)	24 (86)	24 (89)	0.99
Anti-viral				
Ganciclovir	4 (7.3)	3 (11)	1 (3.7)	0.60
Valaciclovir	54 (98)	28 (100)	26 (96)	0.50
Anti-fungal				
Fluconazole	13 (24)	6 (21)	7 (26)	0.70
Caspofungine	10 (18)	7 (25)	3 (11)	0.30
Micafungine	1 (1.8)	1 (3.6)	0 (0)	0.99
Posaconazol	4 (7.3)	3 (11)	1 (3.7)	0.60
Voriconazole	9 (16)	8 (29)	1 (3.7)	0.025

Continuous variables are described with median and interquartile range and compared with bilateral Wilcoxon test. Fisher's test was used to compare frequencies. GVHD: graft-versus-host disease.

Table S3. Clinical characteristics of patients according to enterotype trajectories (among patients with at least two samples studied), related to Figure 2

n patients=	Changed enterotype 22	Identical enterotype 16	p-value
Gender, n Female (%)	5 (23%)	6 (38%)	0.50
Age (years)	57 (46-63)	54 (41-63)	0.7
Group of treatment, n (%)			
Placebo	13 (59%)	6 (38%)	0.2
Azithromycin	9 (41%)	10 (62%)	
Diagnosis, n (%)			
Acute myeloid leukemia	4 (18%)	5 (31%)	0.50
Acute lymphoid leukemia	4 (18%)	3 (19%)	
Myelodysplastic neoplasm	7 (32%)	1 (6.2%)	
Non-Hodgkin lymphoma	2 (9.1%)	3 (19%)	
Hodgkin lymphoma	1 (4.5%)	0 (0%)	
Chronic lymphoid leukemia	1 (4.5%)	2 (12%)	
Chronic myeloid leukemia	1 (3.6)	0 (0)	
Other malignancies	2 (9.1%)	3 (19%)	
Disease status at transplant, (%)			
CR1	3 (14%)	6 (38%)	0.30
>CR1	8 (36%)	5 (31%)	
Other	11 (50%)	5 (31%)	
Donor type, n (%)			
Related	7 (32%)	7 (44%)	0.50
Unrelated	15 (68%)	9 (56%)	
CMV serology (Donor/Recipient), n (%)			
-/-	5 (23%)	4 (25%)	>0.9
-/+	5 (23%)	3 (19%)	
+/-	4 (18%)	3 (19%)	
+/+	8 (36%)	6 (38%)	
Source of stem cells, n (%)			
Peripheral blood	20 (91%)	14 (88%)	>0.9
Bone marrow	1 (4.5%)	1 (6.2%)	
Cord blood	1 (4.5%)	1 (6.2%)	
Conditioning regimen, n (%)			
Myeloablative	5 (23%)	5 (31%)	0.70
Non-myeloablative	17 (77%)	11 (69%)	
Graft-vs-host disease prophylaxis, n (%)			
Ciclosporin-methotrexate	3 (14%)	6 (38%)	0.15
Ciclosporin-mycophenolate mofetil	17 (77%)	10 (62%)	
Post-transplant cyclophosphamide	0 (0)	0 (0)	
Other	2 (9.1%)	0 (0%)	
Acute GVHD, n (%)			
aGVHD	14 (64%)	8 (50%)	0.30
Death before aGVHD	3 (14%)	6 (38%)	
No event at last follow up	5 (23%)	2 (12%)	
Chronic GVHD, n (%)			
Mild	7 (58%)	7 (78%)	0.70
Moderate	3 (25%)	2 (22%)	
Severe	2 (17%)	0 (0%)	
No event at last follow up	10	7	
Bronchiolitis obliterans syndrome, n (%)			
Yes	1 (100%)	0 (0%)	NA
Relapse at 12 months, n (%)			
Relapse	4 (19%)	8 (57%)	0.020
Death before 12 months	1	2	
No event at 12 months	17 (81%)	6 (43%)	
Nutrition, n (%)			
Enteral	5 (23%)	1 (6.7%)	0.30
Oral	13 (59%)	9 (60%)	
Intravenous	4 (18%)	5 (33%)	
Unknown	0	1	
Antibiotics, (%)			
Beta-lactamin			
Amoxicillin	21 (95%)	15 (94%)	>0.9
Piperacilline-Tazobactam	15 (68%)	12 (75%)	0.7
Glycopeptide			
Vancomycin	9 (41%)	6 (38%)	0.8
Cephalosporin			
Ceftriaxone	1 (4.5%)	0 (0%)	>0.9
Cephazoline	4 (18%)	4 (25%)	0.7
Ceftazidim	6 (27%)	3 (19%)	0.7
Cefotaxime	0 (0%)	0 (0%)	NA
Quinolone			
Ofloxacin	21 (95%)	16 (100%)	>0.9
Levofloxacin	1 (4.5%)	0 (0%)	>0.9
Carbapenem			
Imipenem Cilastin	2 (9.1%)	2 (12%)	>0.9
Sulfonamides			
Sulfamethoxazole Trimethoprim	22 (100%)	16 (100%)	
Nitroimidazole			
Metronidazole	0 (0%)	0 (0%)	NA
Aminoglycoside			
Amikacin	20 (91%)	14 (88%)	>0.9
Anti-viral			
Ganciclovir	2 (9.1%)	2 (12%)	>0.9
Valaciclovir	21 (95%)	16 (100%)	>0.9
Anti-fungal			
Fluconazole	7 (32%)	2 (12%)	0.3
Caspofungine	3 (14%)	3 (19%)	0.7
Micafungine	0 (0%)	0 (0%)	NA
Posaconazol	1 (4.5%)	1 (6.2%)	>0.9
Voriconazole	5 (23%)	2 (12%)	0.7

Continuous variables are described with median and interquartile range and compared with bilateral Wilcoxon test. GVHD: graft-versus-host disease.